

Referring Physician:

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UID:986CE45F-0D9A-44C9-B019-C9B1C56D34A9
TCGA-FY-A3ON-01A-PR

Redacted



DOB: Age: Gender: M

Ref#: Hosp#: Provider Group:

Date of Service: Date Received: Outpatient Case #
Room: Bed: Date Reported:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

THYROID, TOTAL THYROIDECTOMY:

- PAPILLARY THYROID CARCINOMA, CLASSICAL VARIANT.
 - Largest tumor nodule is in right lobe and measures 2.5cm in greatest dimension.
 - Two additional tumor nodules are present in the isthmus and left lobe, respectively.
- MARGINS: NEGATIVE. TUMOR IS 0.1CM FROM CLOSEST MARGIN.
- LYMPH NODES: 1/4 POSITIVE FOR TUMOR.
- TUMOR PRESENT IN A BACKGROUND OF LYMPHOCYTIC THYROIDITIS WITH GERMINAL CENTERS.
- SEE COMMENT.

PATHOLOGY TUMOR STAGING DESCRIPTORS:

Histologic type and grade: Papillary thyroid carcinoma, classical variant.

Primary tumor: pT2 (m).

Regional lymph nodes: pN1.

Distant metastasis: pMX.

Margin status: R0.

Pathologic stage: I.

Lymphovascular invasion: Not identified.

Perineural invasion: Not identified.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Date Reviewed:	1/21/12	

Comment: This is compatible with Hashimoto's Thyroiditis in the appropriate clinical setting.

Tumor Staging Information

Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, 7th Ed, and CAP protocol.

1CD-0-3
carcinoma, papillary, thyroid 8264/3
Site: thyroid, NOS 073.9 lw 2/2/12

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Phone

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This report continues... (FINAL)

Acct No.

Patient Name

Patient: [Redacted]

[Redacted]

Case #:

Procedure:	Total thyroidectomy.
Specimen type:	Complete thyroid.
Specimen integrity:	Intact.
Tumor focality:	Multifocal.
Tumor laterality:	Bilateral.
Histologic type:	Papillary thyroid carcinoma, classical variant.
Lymphovascular invasion:	Not identified.
Extracapsular tumor:	N/A.
Tumor capsular invasion:	Not identified.
Extrathyroidal extension:	Not identified.
LYMPH NODES:	1/4.
MARGIN EVALUATION:	Negative.
Distance to closest margin:	0.1cm.
PATHOLOGIC TUMOR STAGING DESCRIPTORS:	
Primary tumor:	pT2 (m).
Regional lymph nodes:	pN1.
Distant metastasis:	pMX.
Margin status:	R0.
Pathologic stage:	I.
Additional pathologic findings:	Lymphocytic thyroiditis with germinal centers.
Comment:	N/A.

Signed by

Source of Specimen:
Thyroid; Total

Clinical History/Operative Dx:
Thyroid cancer

Gross Description:
The specimen is labeled total thyroid and is received without fixative. It consists of a total thyroidectomy specimen weighing 17 grams. A black suture marks the left lobe. With this orientation the right thyroid lobe measures 3.8 cm from superior to inferior, 2.8 cm from medial to lateral, and 1.5 cm from superficial to deep. The left thyroid lobe measures 3.2 cm from superior to inferior, 2 cm from medial to lateral, and

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FINAL SURGICAL PATHOLOGY REPORT

1.5 cm from superficial to deep. The isthmus measures 2 x 1.8 x 1 cm and there is a small well delineated pyramidal lobe 0.7 cm in maximum dimension. The anterior surface of the right thyroid lobe is inked blue, the anterior surface of the left thyroid lobe is inked green, the anterior surface of the isthmus is inked yellow, and the posterior surfaces are inked black. Each lobe and the isthmus is serially sectioned to reveal a firm relatively well delineated pink to pale red nodule within the central and inferior right thyroid lobe. It measures 2.5 x 1.8 x 1.5 cm. The inferior most medial edge of this nodule is adjacent to the isthmus and shows focal calcification. Representative sections of this mass are obtained for research purposes. The parenchyma of the left thyroid lobe is pale red and has a slightly palpably nodular feel throughout. Sections of the isthmus reveal a second circumscribed 1.2 cm pink-tan nodule within the central isthmus, not involving the pyramidal lobe. Following fixation the entire specimen is submitted as summarized below. Section summary: A1-A9) right thyroid lobe from superior to inferior, A10-A14) isthmus, A15-A21) left thyroid lobe, submitted from superior to inferior (block A9 for decalcification).

Microscopic Description:

Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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Photocopy

Pa

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END OF REPORT (FINAL)